PAM Health at Home
825 Main St Ste 110, Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

ROC Order

Order No: R-25972 **Order Date:** 02/19/2026 01:00 PM

Patient

Keen, Marilyn (MR#: 3070)
1304 Berkman Dr.
Victoria, TX, 77904
(361) 433-2891
DOB: 05/17/1948

Physician

Jorge Armando Diaz MD
2710 Hospital Dr Ste 306
VICTORIA, TX, 77901
(361) 582-5737 **Fax:** (361) 574-7072
NPI: 1306817804

Payer Source: PALMETTO GBA
Primary Diagnosis: S81.801A - Unspecified open wound, right lower leg, initial encounter
Certification Period: 01/21/2026-03/21/2026
Requested Staff: Dye, Jimie - RN

Order Description

SCHEDULE:
SN-SN Frequency: 1W1, 2W2, 1W2 Starting Week of 02/15/2026,
SN PRN Visits/Hours: 3 (Wound care for soilage/dislodgement)
PT-PT Frequency: 1W1 EVAL AND TREAT Starting Week of 02/15/2026,

Medications:
Atorvastatin 10 mg tablet(Route=oral, Frequency=at bedtime, Dosage=10mg, amount Of= 1.00)

Furosemide 20 mg tablet(Route=oral, Frequency=daily, Dosage=20mg, amount Of= 1.00)

Latanoprost 0.005 % eye drops(Route=ophthalmic (eye), Frequency=at bedtime, Dosage=0.005%, amount Of= 1.00)

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Spironolactone 25 mg tablet(Route=oral, Frequency=daily, Dosage=25mg, amount Of= 1.00)

Keppra 500 mg tablet(Route=oral, Frequency=2 times a day, Dosage=500mg , amount Of= 1.00)

Interventions:

BEST PRACTICE FALL PREVENTION
Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION
Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN
Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION
Clinician will perform validated test Braden to assess patient for risk of development of Pressure ulcers.
Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

CARDIOVASCULAR: CHF
Clinician to assess/instruct patient/caregiver Symptoms in Heart failure indicated by clinical heart failure guidelines (including dyspnea, orthopnea, edema, or weight gain).

CHF
Clinician will perform weights each visit and report 3lb. weight gain in 2 days or 5lbs. gain in 7 days to physician.

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Clinician will instruct patient/caregiver to weigh daily report weight gain of 3lbs. in 2 days or 5 lbs. in 7 days, and daily keep log. Clinician to obtain abdominal girth to assess for fluid retention.
SN to instruct pt/cg on possible s/s of CHF such as SOB, increased fatigue or weakness, edema in LE, chest pain, and weight gain of 3lbs/1 day and/or 5lbs/1week.
SN to instruct pt/cg on ways to manage CHF such as taking appropriate medications as ordered, wearing compression stockings, avoiding excess salt,exercise as recommended by MD, and reporting to MD/SN weight gain of 3lbs/1 day and/or 5lbs/1week.
SN to teach patient/cg importance of taking daily weights. Upon awakening each morning, and after using restroom, weigh yourself. Scale should be placed on hard surface. Record weight on monthly calendar. Report to MD/SN weight gain of 3lbs/1day and/or 5lbs/1week.
SN to instruct on edema management

GENITOURINARY
Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
Clinician to assess patient for presence of bowel and bladder incontinence. Instruct patient /caregiver measures to improve incontinence.
Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

WOUND OTHER (STASIS, SKIN TEARS,EDEMA,RASHES,ETC)
Clinician to assess wound, instruct as follows:

RLE:
CLEANSE WITH NS OR WOUND CLEANSER WITH 4X4 GAUZE, PAT DRY WITH 4X4 GAUZE. APPLY TRIPLE ANTIBIOTIC OINTMENT AND COVER DAILY with dry dressing.
CG TO PERFORM ON NON NURSING DAYS.

Home health to provide wound care supplies as ordered. Clinician to instruct patient/caregiver on S/s of infection to report. Clinician to discontinue wound care when wound is healed.

HOSPITALIZATION RISK FACTORS
Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: RLE WOUND, HTN, H/O UTI, CHF, GLAUCOMA, OSTEOARTHRITIS

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Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN
SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
SN to instruct patient/caregiver on Heart Healthy diet.

Goals:

OTHER WOUNDS:(STASIS, SKIN TEARS, EDEMA,RASHES,ETC)
LTG-Patient/caregiver will be able to identify/ recall S/S of infection to report by end of episode.

BEST PRACTICE FALL PREVENTION
LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION

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LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN
LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION
LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

CARDIOVASCULAR: CHF
LTG-Patient/caregiver will receive education on Symptoms indicative of heart failure and will be able to recall S/s of Heart failure and when to report by end of episode.

GENITOURINARY
LTG-Patient will tolerate collection of urine without pain and discomfort throughout episode.

HOSPITALIZATION RISK FACTORS
Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN
Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

Allergies:
Medication Allergies :codeine,morphine,Sulfa (Sulfonamide Antibiotics)

DME:
Wound Supplies,Dressing Supplies,Walker,Other (specify): ALCOHOL WIPES, NS GLOVES, PROBE COVERS, PPE MASK

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Safety measures:
911 Emergency / Protocol,911 for Emergency,Assistance with IADLs,Fall / Safety Precautions,Fall Precautions,Hazard-Free Environment,Infection Control Standard Measures and Precautions,Keep Pathways Clear,Keep Pathways Clear and Safe,Medication Safety / Storage,Safe in Movements, Safe Transfers,Safety in ADLs,Slow Position Change,Standard Precautions/Infection Control,Support During Transfer and Ambulation,Transfer Precautions,Universal Precautions,Use of Assistive Devices,Walker / Cane

Nutritional Requirements:
No Added Salt,Protein Hi Diet,Other (specify): Heart healthy
Prognosis:
Fair
Mental Status:
Oriented,Forgetful
Functional Limitations:
Bowel/Bladder(incontinence),Hearing,Endurance,Ambulation
Activities Permitted:
Exercises Prescribed,Walker,Other: SBA
Diagnosis Codes:
S81.801A - Unspecified open wound, right lower leg, initial encounter
S06.5X9D - Traumatic subdural hemorrhage with loss of consciousness of unspecified duration , subsequent encounter
I11.0 - Hypertensive heart disease with heart failure
I50.30 - Unspecified diastolic (congestive) heart failure
I48.91 - Unspecified atrial fibrillation
S22.000D - Wedge compression fracture of unspecified thoracic vertebra , subsequent encounter for fracture with routine healing
G72.89 - Other specified myopathies
G93.41 - Metabolic encephalopathy
F03.90 - Unspecified dementia, unspecified severity, without behavioral disturbance, psychotic disturbance, mood disturbance, and anxiety

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E87.0 - Hyperosmolality and hypernatremia
D64.9 - Anemia, unspecified
F10.20 - Alcohol dependence, uncomplicated
H40.1130 - Primary open-angle glaucoma, bilateral, stage unspecified
H91.93 - Unspecified hearing loss, bilateral
E78.5 - Hyperlipidemia, unspecified
K21.9 - Gastro-esophageal reflux disease without esophagitis
M19.90 - Unspecified osteoarthritis, unspecified site
R29.6 - Repeated falls
W19.XXXD - Unspecified fall, subsequent encounter
Z86.73 - Personal history of transient ischemic attack (TIA), and cerebral infarction without residual deficits
Z87.440 - Personal history of urinary (tract) infections
Z91.81 - History of falling

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify in my estimation that the patient will need skilled care from home health for the next 60 days.
I attest to the following:
1. Marilyn Keen is under my care.
2. I have received the following patient records from Be Healthy at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)

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c. Home Health Plan of Care
d. And the signature below is my signature
3. Each of these patient records have been incorporated into my patient record.
4. These patients records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.

Comments

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March 02, 2026; 01:49 PM

Physician Signature: **Date:** 03/02/2026